Printable Client Pages

Mental Health Group Therapy Worksheets for Adults · Volume 2

Every client worksheet, the Rotation Passport, both contact-card formats, the skills and session reviews, everyday practice pages, and the topic finder.

Book page numbers are kept at the bottom of each page so the codes and page references match the printed book. Print the folded card at 100%; for larger type, use the full-page contacts.

Staff pages are in a separate file.

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SK-MF-01

Observe and Describe

Entry practice / about 10 minutes

Describe one ordinary moment using words another person could observe.

Section A - Today's Practice (in group)

Something I noticed and described today:

Section B - Daily Log

Day / date	What I noticed (factual)	A judgment I caught myself adding (optional)

C - Looking back

Read one description back. Which words report an observation, and which add a judgment?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Say one observation and one judgment; the facilitator can write them in separate columns.

On another visit: Describe the same event to two people. Which words remain factual in both versions?

Ratings are optional. Skip them or use your own words. They aren’t a test or a diagnosis.

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SK-MF-02

My Grounding Options

Entry practice / about 15 minutes

Find a present-moment cue that fits your senses, comfort and surroundings.

Section A - Today's Grounding Practice

Choose a comfortable option; adapt or skip any sense that is unavailable or uncomfortable. 0 = not helpful; 10 = very helpful.

Technique	Helpful today (0-10 or N/A)
Notice available sights, sounds or textures	
Feel feet or another point of support	
Hold a comfortable textured object	

Section B - My Grounding Menu

Example: At a busy checkout, I can notice my feet and name three colors around me.

Technique I would choose	Where I could use it

C - Looking back

Which option would you choose in a busy place? What would you use if it felt uncomfortable?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Point to a grounding option; demonstrate it only if comfortable, then give a word or rating.

On another visit: Design a grounding option that uses no equipment and fits a place with limited privacy.

Ratings are optional. Skip them or use your own words. They aren’t a test or a diagnosis.

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SK-MF-03

Noticing and Returning Attention

Extended practice / about 10 minutes

Practice returning attention to a chosen cue; wandering is part of the exercise.

Section A - My Chosen Anchor

My chosen external cue, sound, support point, or breath if comfortable:

Section B - Tracking Grid

Day / date	Situation or cue	Noticed or returned?	What helped / support needed

Start briefly with a comfortable external cue. Do not add competing voices or deliberate distractions if they cause confusion or distress.

C - Looking back

What made returning easier today: a different cue, a shorter practice, fewer distractions or support?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Use a finger tap to show noticing drift and returning; a written daily log is optional.

On another visit: Compare two anchors briefly. Choose the one that is easiest to return to today.

Ratings are optional. Skip them or use your own words. They aren’t a test or a diagnosis.

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SK-ER-01

Emotions Urges and Actions

Entry practice / about 10 minutes

Notice what happened, what you felt like doing and what you did.

Section A - Map a Recent Moment

Trigger (what happened):

Emotion + Urge (what I felt, and what it made me want to do):

Action (what I actually did):

Section B - Common Emotion-Urge Pairings (reference)

Emotion	An urge some people notice
Sadness	Withdraw
Anger	Confront / push away
Fear	Avoid / flee
Shame	Hide

These are examples, not rules. Your urge may be different, mixed or hard to name.

C - Looking back

Did following the urge help in that situation? What other response or support was available?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that's too much: an easier option, or someone who can help:

Without writing: Say or draw three symbols: what happened, the urge, and what you chose to do.

On another visit: Find a moment when the urge and action differed. What support made the choice possible?

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SK-ER-02

Check the Facts and Choose a Response

Entry practice / about 15 minutes

Check the situation before choosing opposite action, problem-solving or support.

Section A - My Situation

What happened (observable facts):

My interpretation:

Emotion and intensity (0 = none; 10 = very strong):

Section B - Choosing a Response

Does the emotion fit the facts? [ ] Yes [ ] Partly [ ] No [ ] Unsure Is there a real threat or a boundary I need to protect? [ ] Yes [ ] No [ ] Unsure

Do not use opposite action to ignore danger. If the emotion fits or action is needed, consider problem-solving or seeking support. If the emotion or its intensity does not fit, or acting on the urge would be ineffective, discuss a safe opposite action.

My urge:

My chosen response: [ ] Opposite action [ ] Problem-solving [ ] Support

---

Example: No reply yet does not prove rejection. If safe and useful, I can continue my day rather than repeatedly checking.

C - Looking back

What response fits the facts and protects a real need or boundary? Ask for help if unsure.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Tell the facts aloud; point to opposite action, problem-solving, support, or unsure.

On another visit: Use an example where danger or a boundary mattered. Identify support instead of overriding it.

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SK-ER-03

Routines That Support Me

Entry practice / about 10 minutes

Notice daily routines and conditions that affect how manageable emotions feel.

Section A - Vulnerability Audit

Rate how supportive each area is today: 0 = not supportive; 10 = very supportive; N/A = not relevant. Discuss mixed or unclear ratings with your facilitator.

Factor	Supportive today (0-10 / N/A)
Sleep	
Eating regularly	
Physical activity suited to my health	
Substance-related choices and support	
Ways of managing accumulated stress	

Section B - My One Chosen Factor

An area I can influence, with support if needed:

Example: If regular meals are difficult, my first step might be to ask about affordable food options or plan one accessible snack.

> Do not abruptly change prescribed medicine, alcohol or sedatives from this page. Withdrawal concerns need clinical advice.

C - Looking back

Which area needs a routine change, and which needs resources, health care or an accommodation?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Point to one routine factor; choose supportive, mixed, difficult, or not relevant.

On another visit: Choose one factor limited by money, health, or access. Identify a practical support request.

Ratings are optional. Skip them or use your own words. They aren’t a test or a diagnosis.

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SK-DT-01

Coping Through an Intense Moment

Entry practice / about 15 minutes

Prepare a comfortable coping option for distress that is manageable to practice with.

Section A - My Pause Reminder

Stop > Step back > Observe > Proceed on purpose using a suitable skill.

If I may act on an urge to harm myself or someone else, or cannot stay safe, I use my individualized safety plan and contact urgent support immediately. I do not need to finish this worksheet first.

Section B - My Comfortable Coping Options

Choose an option that fits your health and mobility. Breathe gently; stop if dizzy or more distressed. Any movement should be comfortable and approved by your team when needed.

Option	Would try / order
Notice an external sight, sound or texture	
Gentle paced breathing, if comfortable	
Release comfortable muscle tension	
Safe movement suited to my health	

Person I can ask for support / contact:

C - Looking back

Could you find and use one option when distracted? Choose a backup without waiting to seek urgent help.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Point to a comfortable coping option and practice or observe; passing is allowed.

On another visit: Prepare a backup if the first coping option is unavailable. Urgent help takes priority.

If you can’t stay safe, or someone is in danger right now, get urgent help first. This page isn’t a safety plan.

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SK-DT-02

Acceptance and My Next Choice

Extended practice / about 15 minutes

Acknowledge one manageable fact while keeping protective choices available.

Section A - Fighting and Accepting

Fighting a fact	Accepting a fact
"This should not have happened."	"This happened. What can I do now?"
Replaying "if only..."	Returning to a useful next step

Acceptance is not approval. It does not mean staying in danger, accepting mistreatment, or giving up action you can take.

Section B - My Practice

A manageable fact I am practicing accepting:

My statement: "This is true. I do not have to like it. My useful next step is..."

Choose an ordinary, verified fact. This general practice is not individual trauma or psychosis treatment.

C - Looking back

What fact can you acknowledge, and what action remains yours to take? Acceptance does not require approval.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Say a manageable fact and a choice still available; no personal disclosure is required.

On another visit: Separate what cannot be changed now from one protective or useful action you can take.

Ratings are optional. Skip them or use your own words. They aren’t a test or a diagnosis.

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SK-DT-03

Comfort and Distraction Options

Entry practice / about 15 minutes

Choose sensory comfort or brief distraction that fits your needs.

Section A - My Five Senses Menu

Sense	My item
Sight	
Sound	
Smell	
Taste	
Touch	

Section B - Helpful Now vs. Helpful Later

Helps in the moment	Helps the underlying issue

C - Looking back

A brief distraction sequence I choose, if useful:

Which option helps for now? What still needs problem-solving, practical help or a conversation later?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that's too much: an easier option, or someone who can help:

Without writing: Point to an available sense or describe a comforting object; skip uncomfortable options.

On another visit: Choose a sensory option for a setting where sound, scent, or touch is unavailable.

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SK-CG-01

Situation Thought Feeling Action

Entry practice / about 15 minutes

Map a manageable situation and notice possible links, without forcing it into a label.

Section A - My Chain

Situation:

Thought:

Feeling:

Behavior:

Section B - Possible Thinking Traps

Trap	Example
All-or-nothing	"One mistake means I failed at everything."
Catastrophizing	"This will ruin everything."
Mind-reading	"They must think I am annoying."
Should statements	"I should be over this by now."
Emotional reasoning	"I feel like a failure, so I must be one."

A trap I noticed, if any (none / unsure are also okay):

C - Looking back

Which link could you respond to differently? Which part of the situation needs outside support?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Place or draw four simple symbols for situation, thought, feeling, and behavior.

On another visit: Choose one link you can influence and one circumstance needing practical support.

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SK-CG-02

A Fuller Thought Record

Entry practice / about 15 minutes

Look for a fairer, more complete view using the facts available.

Section A - My Original Thought

Thought:

Believability before: ___ /10 (0 = not at all; 10 = completely)

Section B - Evidence

Evidence for	Evidence against / other facts

Example: Thought: "I never finish anything." Facts: I postponed one task; I did complete a smaller one. Alternative: "Some tasks are hard to start. I can try one manageable step."

Real discrimination, abuse and material hardship are not thinking traps to dismiss. Do not debate delusions or hearing voices in this group exercise.

C - Looking back

A fuller thought that fits the facts:

Optional: original thought before ___ /10; after ___ /10. 0 = not believable; 10 = completely believable. The rating may stay the same.

What useful action could you take even if the original thought still feels believable?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Talk through facts for and against a thought; ask the facilitator to model the structure.

On another visit: If the thought remains believable, add missing context rather than forcing a positive answer.

Ratings are optional. Skip them or use your own words. They aren’t a test or a diagnosis.

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SK-CG-03

Making Room Around a Thought

Extended practice / about 10 minutes

Try noticing a manageable thought as a thought, then choose what to do.

Section A - My Looping Thought

Choose a thought that feels manageable to practice with:

Section B - Trying Some Distance

"I am having the thought that..."

You can try "My mind is telling me..." or say the phrase gently. Keep only what feels useful. The exercise does not decide whether the thought is true.

> Use a manageable thought or a neutral example; this general exercise is not treatment for hearing voices or psychosis.

C - Looking back

Did the phrase help, irritate you or make no difference? Choose whether to keep it, change it or stop.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Say a distancing phrase once or listen to the facilitator model it; writing is optional.

On another visit: Compare a distancing phrase with naming the thought briefly; choose the more useful option.

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SK-BA-01

Activities Mood and Energy

Entry practice / about 15 minutes

Notice patterns and plan an activity that fits your health and circumstances.

Section A - A Recent Day

Mood: 0 = very low; 10 = very positive. Energy: low / medium / high.

Time	What I did	Mood / energy
Morning		
Afternoon		
Evening		

Section B - Pleasure and Accomplishment

Enjoyable or meaningful options	Options giving a sense of accomplishment

Example: On a low-energy day, listening to one song or putting away one item can be a meaningful start.

> Match activity to your health and energy. Very little sleep with unusually high energy needs prompt clinical review, not more activation.

C - Looking back

What made an activity worthwhile, tiring or hard to access? Use that information in your next plan.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Name one activity and point to pleasure, accomplishment, both, neither, or unsure.

On another visit: Plan an activity that costs nothing and fits your health, energy, and access today.

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SK-BA-02

A Value and a Small Action

Extended practice / about 15 minutes

Connect a personally meaningful direction to an action you can realistically choose.

Section A - My Value(s) Right Now

A direction that matters to me today (my own word is welcome):

Reference: connection / growth / honesty / health / creativity / contribution / spirituality/meaning

B - A task and the direction it could serve

Avoided task:

Connects to value:

A delay can protect health or safety, or reflect a missing resource. Choose a different action or ask for support when that fits better.

C - Looking back

What is the smallest useful action? If the task protects you from harm or overload, what support fits instead?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Point to a value and say one small action; a drawing or one word can be enough.

On another visit: Keep the same value but choose an action for a day with less energy or fewer resources.

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SK-BA-03

My Task Steps and Reminder

Entry practice / about 15 minutes

Break a task into usable steps and choose a reminder you are likely to notice.

Section A - My Task and Step Ladder

The task I want to work toward:

Step	A manageable part of the task
1 - smallest starting move	
2	
3	
4	
5 - full task	

Example: Task: arrange a follow-up visit. Step 1: find the clinic number. Step 2: write one question. Step 3: make the call or ask for help calling.

Section B - My Next Step and Barrier

I will do ___ at / after ___.

What might block it? Time / health / money / tools / attention / support / other:

C - Looking back

Choose only the next step for now. What would help you restart after an interruption?

D - My next step

A question, a support request, or nothing to add today.

Without writing: Say the first two steps aloud, or arrange two drawn boxes in order.

On another visit: Build a restart step for after an interruption; the ladder does not have to begin again.

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SK-IE-01

Making a Clear Request

Entry practice / about 15 minutes

Practice a brief request for a situation where speaking up is safe.

Fictional example: "When plans change, please text me before I leave. That helps me arrange the trip." Keep your own wording; a clear request does not make an unsafe relationship safe.

A - My four-part request

Describe the situation: When ...

Express my view or feeling: I ... because ...

Make my request: I would like ...

Explain why it matters: This would help because ...

Section B - Style Check (reference)

Passive	Assertive	Aggressive
Not stating the need	Stating it clearly and respectfully	Disregarding the other person

This is a four-part request practice, not every part of DEAR MAN. Directness, wording and whether to speak at all should fit safety, culture and circumstances.

C - Looking back

If the answer is no, what can you clarify, negotiate or decide next? A clear request cannot guarantee agreement.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Rehearse a short request aloud using a fictional situation; the facilitator may prompt you.

On another visit: Prepare one respectful response if the request is declined; consider safety and alternatives.

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SK-IE-02

A Boundary I Can Keep

Extended practice / about 15 minutes

Consider another perspective when useful, while keeping a boundary about your own choices.

Section A - My Situation

Who / what (general terms are okay):

If setting a boundary could bring threats, violence or other serious harm, plan privately with your clinician before using this script. You do not have to validate harmful behavior.

Section B - My Response

Acknowledge the perspective, if appropriate:

State my boundary clearly:

C - Looking back

What will you do if the boundary is ignored? Plan privately if speaking up could bring harm.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Choose a sample boundary and practice one sentence aloud, or listen and point.

On another visit: Identify a safe limit you can control and who can support it if the other person disagrees.

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SK-IE-03

Saying No and Understanding Guilt

Entry practice / about 10 minutes

Practice declining a request and consider what guilt may mean.

Section A - My Situation

What am I being asked, and by whom?

Section B - My Brief No

A respectful decline I could use:

Example: "I cannot take that on today." You may offer an alternative only if you want to and it is realistic.

> If declining could put you at risk, plan privately with your clinician instead of testing the script in that situation.

C - Looking back

Is there a repair you want to make, discomfort you can allow, or pressure that calls for support?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Practice a one-sentence decline or point to a sample phrase; a personal example is optional.

On another visit: Try a shorter decline that does not over-explain; distinguish discomfort from an unsafe situation.

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PE-01

Depression and Withdrawal

Entry practice / about 10 minutes

Explore a common cycle of low mood and reduced activity without using it to diagnose.

Section A - Common Patterns

Mood: low, numb or irritable. Body: changes in sleep, appetite or energy. Mind: concentration difficulties or self-critical thoughts. Behavior: withdrawal or reduced activity.

A possible withdrawal cycle: low mood > less activity or connection > fewer rewarding experiences > lower mood. This is one pattern, not everyone's experience.

Section B - Private Self-Check

Mood:

Body:

Mind:

Behavior:

C - Looking back

Where could a small activity, connection or support request make the cycle more manageable?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Point to one part of the withdrawal cycle and name a manageable action or support.

On another visit: Look for one exception to the withdrawal cycle and a condition that made it possible.

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PE-02

Anxiety and the Alarm Response

Entry practice / about 10 minutes

Notice how anxiety can involve thoughts, sensations and actions.

Section A - What My Body Does

Check any that apply to you: [ ] Racing heart [ ] Fast breathing [ ] Tight chest [ ] Restlessness [ ] Muscle tension [ ] Stomach upset

Section B - Helpful vs. Excessive Worry

A time worry felt helpful	A time worry felt excessive

New, severe or unusual physical symptoms need medical attention. Do not assume they are anxiety.

C - Looking back

Is this a familiar pattern or something new that needs medical advice? A body signal alone cannot decide.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Point to a body signal or name it generally; you do not need to describe a private event.

On another visit: Separate a problem needing action from worry needing support; seek medical advice for new symptoms.

Ratings are optional. Skip them or use your own words. They aren’t a test or a diagnosis.

Worry can signal a real danger or unmet need. Ask what practical protection or support is needed; do not assume every anxious prediction is a thinking error.

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PE-03

Stress Demands and Recovery

Entry practice / about 10 minutes

Look at current demands and realistic opportunities for rest, support or change.

Section A - My Current Stress Sources

Section B - My Current Recovery Practices, If Any

Possible supports: a brief pause, comfortable external grounding, help with a demand, or an evening routine that fits your situation.

C - Looking back

Which demand can you influence? Which needs practical help, protection or a change outside your control?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Draw or describe one demand and one recovery opportunity; the facilitator can note them.

On another visit: Identify one demand you cannot remove and one support or boundary that could reduce its load.

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PE-04

Understanding a Panic Cycle

Extended practice / about 10 minutes

Map a familiar or fictional panic cycle without deliberately creating symptoms.

This page maps a panic cycle; do not deliberately bring on symptoms or use it to test whether symptoms are dangerous. Stop unwanted body focus. New, severe or unusual physical symptoms need medical attention; do not assume they are panic.

Section A - My Panic Cycle Map

Sensation > Interpretation > Fear > Intensified sensation

1. Sensation:

---

2. Interpretation:

---

3. Fear:

---

4. Then:

Section B - My Interruption Strategies

[ ] Grounding [ ] Comfortable paced breathing [ ] A realistic coping statement [ ] Other:

C - Looking back

Choose a response with your clinician. What symptoms or uncertainty would make medical help the priority?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Use a fictional example to say the links in a panic cycle; observe or pass if preferred.

On another visit: discuss a helpful response with your team.

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PE-05

Sleep Habits and Mood

Entry practice / about 10 minutes

Choose a realistic sleep-related habit or question for your treatment team.

Section A - Sleep Habits Audit

Mark one you already do well (yes) and one you're willing to adjust (>): [ ] Consistent wake time [ ] Limiting screens before bed [ ] Caffeine timing [ ] A wind-down routine [ ] A sleep setting that is feasible for me

Section B - What Gets in the Way

Answer:

> Very little sleep with unusually high energy, elevated mood, agitation or impulsivity needs prompt clinical review, rather than simply another sleep-habit exercise.

C - Looking back

What ongoing sleep problem needs a care conversation? These habits are not a complete insomnia treatment.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Point to one sleep habit and describe a barrier; no sleep diary is required today.

On another visit: Adapt one habit for shift work, caregiving, housing, or health; persistent insomnia needs care.

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PE-06

Strong Emotions and a Pause

Entry practice / about 10 minutes

Notice a moment when strong emotion made reflection harder.

Section A - My Earliest Warning Sign

An early clue I might notice, or an outside cue I could use:

Section B - A Recent Example

A manageable or fictional situation, and my response:

C - Looking back

What could cue a pause if you do not notice an early body signal? Choose an external reminder if useful.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Point to a pause strategy or give a brief word for an early sign; noticing a sign is optional.

On another visit: Plan a cue from your environment when internal warning signs are hard to notice.

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PE-07

A Guide to Feeling Able to Engage

Extended practice / about 15 minutes

Use a three-zone teaching model to describe experience and access needs.

Section A - A Three-Zone Teaching Guide

Keyed up or overwhelmed: sometimes called above the window.

Able to engage: sometimes called within the window.

Flat, tired or disconnected: sometimes called below the window. These descriptions do not measure a biological state.

When I feel keyed up or overwhelmed:

When I feel flat, tired or disconnected:

Section B - My Matched Strategies

When keyed up or overwhelmed	When flat, tired or disconnected

C - Looking back

What makes participation easier today? This model and any rating do not measure your nervous system.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Point to a zone in the teaching model or draw one; it is not a measured body state.

On another visit: Name a condition that helps you feel more able to engage, without trying to force a zone change.

Ratings are optional. Skip them or use your own words. They aren’t a test or a diagnosis.

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PE-08

Trauma Reminders and Present Support

Extended practice / about 10 minutes

Consider present-focused support without describing a traumatic event.

> Use a general or fictional example. No trauma details are needed. Stop and ask for support if this becomes distressing.

Section A - What a Trigger Is

A reminder can bring up distress linked with an earlier experience. A present-day stress response does not by itself establish trauma or PTSD.

Section B - My Trigger Categories, If Relevant (general only)

[ ] Sounds [ ] Situations [ ] Tone of voice [ ] Crowds [ ] Other (no need to describe the underlying event)

C - Looking back

Choose a pause signal or support request. You can ask for help without explaining the past event in group.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Name a present-day support or grounding choice; do not describe trauma details.

On another visit: Choose a way to ask for a pause or private support when a topic feels uncomfortable.

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PE-09

A Fairer Response to Self Criticism

Extended practice / about 15 minutes

Try a response that acknowledges difficulty without judging your whole self.

Section A - Guilt vs. Shame

Guilt may concern a specific action; shame may involve judging the whole self. Neither feeling proves blame.

Is what you're feeling bad about closer to guilt or shame?

Section B - My Self-Compassion Break

A recent self-critical thought:

Self-kindness (what would I say to a friend?):

Common humanity (how is this part of being human?):

Noticing the feeling without letting it define my whole self:

C - Looking back

Does the kinder response feel believable? Make it more specific or modest rather than forcing reassurance.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Listen to or say one fair, kind sentence; a fictional example is fine.

On another visit: Make the compassionate statement more believable by making it specific and modest.

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PE-10

Values and Everyday Choices

Entry practice / about 10 minutes

Name a meaningful direction and an everyday action that could express it.

Section A - The Values Compass (reference)

Relationships / Health / Work/growth / Creativity / Contribution / Spirituality/meaning

Section B - My Value(s) Right Now

Answer:

What living this value looks like, even small:

C - Looking back

What would this value look like on a difficult day with limited energy or resources?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Point to a value word or draw something that matters; no explanation is required.

On another visit: Choose a meaningful action that fits a difficult day rather than an ideal day.

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PE-11

Understanding Anger and Responding

Entry practice / about 15 minutes

Explore an anger pattern and a response suited to facts and safety.

Section A - What Might Anger Be Signaling?

Anger can relate to a perceived threat, an unmet need, stress, hurt, unfairness or a boundary. It does not by itself prove that someone else acted wrongly.

What might be involved for me, if anything?

Section B - My Anger Cycle

Trigger - what happened?

Physical signs:

Urge:

Action:

C - Looking back

What needs attention: a problem, an injustice, a boundary, a pause or support? More than one may fit.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Point to an early sign of anger and say one safe response or support option.

On another visit: Separate the boundary or problem from the anger urge; identify a safe way to address it.

Ratings are optional. Skip them or use your own words. They aren’t a test or a diagnosis.

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PE-12

Grief Loss and Support

Extended practice / about 15 minutes

Explore a loss only as far as feels useful today.

Section A - Types of Loss (reference)

A relationship ending / a job or role lost / a diagnosis or health change / a version of oneself / an imagined future / a home or community left behind

Section B - My Loss and My Feelings About It

A loss I'm grieving:

Feelings that have shown up: [ ] Sadness [ ] Relief [ ] Guilt [ ] Anger [ ] Numbness [ ] Other:

C - Looking back

What support would you welcome or decline? You do not need to follow a timetable or find a lesson in the loss.

D - My next step

A question, a support request, or nothing to add today.

Without writing: Use a symbol or general word for a loss; listening or passing is a valid choice.

On another visit: Name a routine or support that can coexist with grief; there is no required grief timeline.

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PE-13

Loneliness and Connection

Entry practice / about 10 minutes

Consider the kind of contact, company or solitude you want.

Section A - Solitude vs. Isolation Self-Check

[ ] Comfortable solitude [ ] Unwanted isolation [ ] Mixed [ ] Unsure

Section B - My Connection Step

Who:

What I'll do:

By when:

Connection ideas: a text or brief call to a trusted, safe contact, or a low-pressure group activity.

C - Looking back

What makes the contact realistic and welcome? Choose a backup if the first person or activity is unavailable.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Point to a type of connection or say one low-pressure option; names are optional.

On another visit: Plan a connection that does not require money, transport, or a long conversation.

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PE-14

Boundaries in Relationships

Extended practice / about 15 minutes

Explore limits in the context of safety, culture, responsibilities and dependence.

Section A - Boundary Styles (reference)

Few limits; very firm limits; or flexible limits with trust. Firm boundaries may be protective in unsafe situations.

Which feels closest to your usual style?

Section B - My Relationship and Boundary

Relationship:

Boundary I'm considering:

Anticipated pushback and how I'll understand it:

> Threats, stalking, coercion or violence are not ordinary pushback to normalize. Seek private support before trying a boundary that may bring harm.

C - Looking back

What choice can you control? What practical or professional support is needed before acting on it?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Say one boundary sentence for a fictional relationship; no identifying details are needed.

On another visit: Choose a boundary about your own action and a support option if stating it is unsafe.

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PE-15

When Starting a Task Is Hard

Entry practice / about 10 minutes

Find the barrier before deciding that a task needs more effort.

Section A - My Stuck Task

What am I trying to do?

Section B - What Is Getting in the Way?

[ ] Mood [ ] Task size [ ] Unclear instructions [ ] Sleep [ ] Pain / health [ ] Attention [ ] Money / tools [ ] Caring responsibilities [ ] Time / transport [ ] Missing support [ ] Other / unsure

The main barrier today:

C - Looking back

Does the next move need a smaller step, clearer instructions, an accommodation or a resource?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Talk through a task using one action word at a time; the facilitator can draw the steps.

On another visit: Identify whether the obstacle is clarity, energy, emotion, resources, or access; adapt accordingly.

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PE-16

Support Roles and Help Routes

Entry practice / about 15 minutes

Distinguish everyday company, direct support and urgent professional help.

Section A - Layered Support Map

Inner circle - closest personal supports:

Middle circle - community, peers and groups:

Outer circle - professional and crisis resources:

Section B - Crisis Resources (to be completed with your program - local information)

Local/national crisis line:

My program's after-hours contact:

A trusted personal contact:

C - Looking back

Which route fits which need? Verify details privately, including what to do if there is no answer.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Point to a support role; arrange private help to verify contact details if writing is difficult.

On another visit: Choose a backup when a contact does not answer; urgent danger requires immediate help.

If you can’t stay safe, or someone is in danger right now, get urgent help first. This page isn’t a safety plan.

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PE-17

Setbacks and Earlier Support

Extended practice / about 15 minutes

Explore a setback using definitions agreed with your care team.

Section A - Lapse vs. Relapse (reference)

Some care plans distinguish a brief setback from a fuller return of difficulties. The meaning depends on the condition and care plan; ask your team.

Section B - My Early-Warning Sign

Answer:

My Course-Correction Plan

If I notice a setback, my next support or care step is:

C - Looking back

What sign or change would prompt you to ask for help sooner? Do not wait for the situation to become a relapse.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Name a general early sign and a support action; details can remain private.

On another visit: Identify an early sign another trusted person could notice and how you want support offered.

Lapse and relapse are working terms to discuss with your team. Their meaning differs across conditions and care plans; this page does not classify your experience or determine risk.

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PE-18

Follow Up After Stepping Down

Entry practice / about 15 minutes

Review practical follow-up arrangements with your team.

Section A - Skills and Support I Will Keep

One useful skill and a realistic place to practice it:

A person or service I can contact:

Section B - My Follow-Up

Provider / service and appointment date:

Appointment confirmed? [ ] Yes [ ] Not yet Access: [ ] Transport / connection arranged [ ] Cost clarified [ ] Help needed

If the appointment is delayed or I cannot attend, I will contact:

C - Looking back

Early warning sign / support route / availability of bridge support to verify privately:

What is confirmed, what is still a referral, and who will act if there is a delay or access problem?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Say one follow-up need and one access barrier; ask for a private planning conversation.

On another visit: Make a backup plan if the first follow-up appointment or transport arrangement falls through.

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PR-01

Arriving

Entry practice / about 10 minutes

Choose how much you want to bring into the room today.

A - A way into the conversation

What would make today worth staying for, even a little?

B - Reflection

What brought you here, in your own words:

One hope you have for your time here:

C - Looking back

What would help you take part today: listening, a question, a brief contribution, an adaptation or a private check-in?

D - What I want to keep or discuss

A thought, question, or private follow-up I want to carry forward (optional):

Without writing: Use one word, a gesture, or a drawing for how you arrive; listening and passing are welcome.

On another visit: Notice one condition that would help you participate today; you may keep it private.

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PR-02

Carrying the Week

Entry practice / about 10 minutes

Notice what is taking your attention and what kind of support you want.

A - A way into the conversation

What needs attention before you can join this conversation?

B - Reflection

What I'm carrying in from outside this room:

What it might feel like to set it down, even briefly:

C - Looking back

Would you like to keep reflecting, ask for practical help, take a pause or leave the topic for a private conversation?

D - What I want to keep or discuss

A thought, question, or private follow-up I want to carry forward (optional):

Without writing: Show heavy, light, or mixed with a gesture or symbol; explaining the story is optional.

On another visit: Notice whether you want company, practical help, a pause, or privacy with what you carry.

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PR-03

The Mask and the Mirror

Entry practice / about 10 minutes

Explore what you choose to show or keep private in different settings.

A - A way into the conversation

Would you rather describe how you seem, how you feel, or pass?

B - Reflection

One place I wear a "mask":

What that mask might be protecting:

A place where I feel a little less need for it:

C - Looking back

What choice about openness protects you or fits your values? More disclosure is not automatically better.

D - What I want to keep or discuss

A thought, question, or private follow-up I want to carry forward (optional):

Without writing: Use a fictional example or a symbol for what you show and keep private.

On another visit: Notice a boundary that lets you be more authentic without sharing more than you want.

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PR-04

Frustration and Stuckness

Entry practice / about 10 minutes

Explore a difficulty without assuming that being stuck means not trying.

A - A way into the conversation

What has felt slow or frustrating lately?

B - Reflection

What "stuck" feels like for me:

What practical barrier is present, and what support might help?

C - Looking back

What would help most now: information, time, a smaller step, a resource, being heard or something else?

D - What I want to keep or discuss

A thought, question, or private follow-up I want to carry forward (optional):

Without writing: Name or point to one area of stuckness; no explanation is required.

On another visit: Identify whether you need a smaller action, more information, a resource, or simply time.

Ratings are optional. Skip them or use your own words. They aren’t a test or a diagnosis.

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PR-05

Trust and Safety Here

Entry practice / about 10 minutes

Choose your participation boundary; concerns about safety can be discussed privately.

A - My participation choice

[ ] Listen [ ] Use a general example [ ] Speak briefly [ ] Ask for a private check-in [ ] Pass

B - For my own use or a private conversation

A condition or adaptation that would make participation easier:

A concern or request I want to take privately to staff (optional):

C - What I want to request

Is there a group agreement or facilitator action you want to request? A concern does not have to become a public discussion.

D - My preferred follow-up

Who I want to speak with, how and when (optional):

Without writing: Ask for a private check-in, or point to a participation choice. You do not have to rate safety publicly.

On another visit: Review whether a requested agreement or adaptation helped; you may keep the answer private.

> Group members are asked to protect privacy, but the program cannot guarantee what other members do. Ask staff about confidentiality limits and safeguarding duties.

Ratings are optional. Skip them or use your own words. They aren't a test or a diagnosis.

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PR-06

The Inner Critic

Entry practice / about 10 minutes

Explore a self-critical thought or a fictional example.

A - A way into the conversation

Would a neutral example or your own self-critical phrase be easier today?

B - Reflection

Something my inner critic commonly says (a summary is fine):

Would I say this to a friend? What would I say to them instead?

C - Looking back

What response feels fair enough to keep? If this concerns hearing voices, ask for an individual clinical conversation.

D - What I want to keep or discuss

A thought, question, or private follow-up I want to carry forward (optional):

Without writing: Say a general critical phrase or draw a volume dial; personal details are optional.

On another visit: Notice a time you responded to the critic differently, even if the thought remained.

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PR-07

Grief Loss and What Changed

Extended practice / about 10 minutes

You can reflect generally, listen or pass; the loss does not need to be explained.

A - A way into the conversation

Would you prefer listening, general reflection, or sharing a little today?

B - Reflection

A loss I'm carrying, tangible or not:

What kind of loss feels most present for me right now:

C - Looking back

What would help after this conversation? You may want company, routine, quiet, practical support or private care.

D - What I want to keep or discuss

A thought, question, or private follow-up I want to carry forward (optional):

Without writing: Choose a general word, symbol, or silence for a loss; you may pass entirely.

On another visit: Notice what support feels welcome now and what advice you do not want.

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PR-08

Hope and Doubt

Entry practice / about 10 minutes

Explore hope, doubt or uncertainty without choosing a preferred feeling.

A - A way into the conversation

What are you uncertain about, without having to sound hopeful?

B - Reflection

What keeps a flicker of hope alive for me, even alongside doubt:

What might my doubt be telling me, and what support would help?

C - Looking back

What question or concern deserves an answer? No hopeful conclusion is required before asking for help.

D - What I want to keep or discuss

A thought, question, or private follow-up I want to carry forward (optional):

Without writing: Point to hope, doubt, both, or unsure; no explanation is required.

On another visit: Name a small reason to keep engaging without needing to feel hopeful first.

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PR-09

Connection and Isolation

Entry practice / about 10 minutes

Explore when contact or solitude is welcome, possible and safe.

A - A way into the conversation

What kind of contact feels manageable today, if any?

B - Reflection

What my withdrawal pattern usually looks like, if I have one:

Is there one person I could imagine reaching toward, even briefly, this week?

C - Looking back

What would make a connection feel more manageable? Deliberate solitude may also fit your needs today.

D - What I want to keep or discuss

A thought, question, or private follow-up I want to carry forward (optional):

Without writing: Point to alone, connected, both, or unsure; use a general example if useful.

On another visit: Notice what makes connection feel safer or more manageable; sharing names is unnecessary.

Ratings are optional. Skip them or use your own words. They aren’t a test or a diagnosis.

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PR-10

Resilience and Quiet Wins

Entry practice / about 10 minutes

Notice an effort or a condition that helped, without comparing it with anyone else's.

A - A way into the conversation

What took effort this week that someone else might miss?

B - Reflection

A "quiet win" from this past week - something I might normally dismiss as not a big deal:

Do I tend to notice my own small wins, or do they usually go unnoticed?

C - Looking back

What support, opportunity or adjustment helped that effort? What could make it available again?

D - What I want to keep or discuss

A thought, question, or private follow-up I want to carry forward (optional):

Without writing: Say one quiet win or use a symbol for effort; comparing with others is unnecessary.

On another visit: Name a support or circumstance behind the win, so progress is not reduced to willpower.

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PR-11

Giving and Receiving Feedback

Extended practice / about 10 minutes

Choose whether you want feedback, listening or neither.

Ask whether feedback is wanted first. Describe one specific behavior. Either person can decline or stop; ask the facilitator for help.

A - A way into the conversation

Would you like listening, feedback, or neither today?

B - Reflection

My instinct when I receive feedback - defend, shut down, something else:

Is there a difference between feedback that feels critical and feedback that feels like care?

C - Looking back

What feedback would you use, decline or ask to clarify? Ask permission before offering feedback to someone else.

D - What I want to keep or discuss

A thought, question, or private follow-up I want to carry forward (optional):

Without writing: Use a fictional example to practice asking permission before giving feedback.

On another visit: Practice describing an observation and impact without assigning a motive to someone else.

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PR-12

Anger and Boundaries

Entry practice / about 10 minutes

Explore anger without assuming it proves who was right or wrong.

> No confrontation is required. Pause and ask the facilitator for support if the conversation becomes threatening or overwhelming.

A - A way into the conversation

What helps you notice anger before acting on it?

B - Reflection

What might this anger be signaling, and what response would be useful and safe?

Was I taught that anger itself is bad or dangerous? Where did that come from?

C - Looking back

What limit or concern needs attention? Choose a safe way to respond or ask for help.

D - What I want to keep or discuss

A thought, question, or private follow-up I want to carry forward (optional):

Without writing: Name an emotion or draw a boundary; a private example may stay private.

On another visit: Notice a way to protect a limit without escalating a conflict; ask for support if unsafe.

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PR-13

Shame and Self Forgiveness

Extended practice / about 10 minutes

Separate a specific responsibility from a judgment about your whole worth.

A - A way into the conversation

Would you rather use a fictional example or reflect privately?

B - Reflection

Guilt can concern a specific action. Shame can become a judgment of the whole self. Feelings alone do not establish that you caused harm.

Am I judging my whole self, or identifying a specific action?

If I caused harm, what realistic repair is mine to make? If I did not, what support do I need?

C - Looking back

What is yours to repair, if anything? What belongs to someone else or needs protection and support?

D - What I want to keep or discuss

A thought, question, or private follow-up I want to carry forward (optional):

Without writing: Point to responsibility, self-judgment, or unsure; a fictional example is okay.

On another visit: Separate a repair you can make from responsibility that belongs to someone else.

Ratings are optional. Skip them or use your own words. They aren’t a test or a diagnosis.

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PR-14

Identity Beyond the Diagnosis

Entry practice / about 10 minutes

Choose a role, interest or part of yourself you want to make room for.

A - A way into the conversation

What part of your identity would you like acknowledged today?

B - Reflection

A part of my identity beyond a diagnosis, if I have one, or beyond being in treatment:

Has treatment started to feel like it's taking over how I see myself? In what way?

C - Looking back

Does a diagnosis feel useful, limiting, mixed or irrelevant to this reflection? You do not have to distance yourself from it.

D - What I want to keep or discuss

A thought, question, or private follow-up I want to carry forward (optional):

Without writing: Name a role, interest, or strength, or draw a symbol; you do not need to share a diagnosis.

On another visit: Choose one identity or role you want to give more room to, at your own pace.

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PR-15

Navigating Outside Relationships

Entry practice / about 10 minutes

Explore a relationship without identifying the other person.

A - A way into the conversation

Is there a relationship question you want to explore generally today?

B - Reflection

How has being in this program affected one specific relationship in my life?

Is there a boundary or conversation that feels worth having with someone right now?

C - Looking back

What interaction or boundary is within your control? What support would make a conversation safer or more useful?

D - What I want to keep or discuss

A thought, question, or private follow-up I want to carry forward (optional):

Without writing: Use a general relationship type or fictional example; identifying details are unnecessary.

On another visit: Notice one interaction pattern and one choice you can control, with support if needed.

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PR-16

Setbacks and Recovery Momentum

Entry practice / about 10 minutes

Look at a setback without needing to turn it into a positive story.

A - A way into the conversation

Would it help to talk about a setback or a next step?

B - Reflection

What did I make my setback mean about myself or my progress overall?

What's one next step, regardless of how the setback happened?

C - Looking back

What information can you use now? What practical help or change would support the next decision?

D - What I want to keep or discuss

A thought, question, or private follow-up I want to carry forward (optional):

Without writing: Use a word or symbol for a setback; details and public sharing are optional.

On another visit: Notice a supportive response you could use even when motivation is low.

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PR-17

Preparing for Transition

Entry practice / about 10 minutes

Explore feelings and practical questions about a possible change in care.

A - A way into the conversation

What is one question you have about leaving this level of care?

B - Reflection

What feels most uncertain to me about eventually stepping down from this level of care?

What's one support I already have in place, or could put in place, for that transition?

C - Looking back

Which concern needs someone to listen, and which needs an appointment, information or an access task?

D - What I want to keep or discuss

A thought, question, or private follow-up I want to carry forward (optional):

Without writing: Point to a transition concern or name it generally; request private planning if useful.

On another visit: Separate an emotional concern from an access task and choose the kind of help each needs.

Ratings are optional. Skip them or use your own words. They aren’t a test or a diagnosis.

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PR-18

Endings Goodbyes and Continuity

Entry practice / about 10 minutes

Choose whether and how to reflect on an ending.

A - A way into the conversation

Would you prefer a goodbye, a brief acknowledgment, or quiet reflection?

B - Reflection

What's my own history with endings and goodbyes - do they tend to feel easy or hard for me?

What would I want to say to someone leaving this group, even if it's not today?

C - Looking back

What would you like to carry forward? Goodbye messages and public sharing are optional.

D - What I want to keep or discuss

A thought, question, or private follow-up I want to carry forward (optional):

Without writing: Use a symbol or general word for an ending; you may listen or pass.

On another visit: Notice what you want to carry forward and how continuity could be supported outside this group.

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PH-WS-01

Daily Rhythm Anchors

Entry practice / about 15 minutes

Notice a few everyday cues and how they fit your health and circumstances.

Section A - Daily Rhythm Cues

Everyday routines can provide useful cues. Choose an anchor with your team that fits work, caregiving, health and access; do not change prescribed sleep or light treatment from this log.

Section B - 3-Day Log

Day	Wake time	Daytime routine cue	Meals (roughly)	Social contact
1				
2				
3				

My chosen anchor to stabilize:

C - Looking back

Which anchor could stay flexible on a disrupted day? Review sleep or mood concerns with your team.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Say the three daily anchors aloud or point to a routine; exact times are optional today.

On another visit: Adapt one anchor for a day when caregiving, work, housing, or health changes the schedule.

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PH-WS-02

Eating Opportunities and Energy

Entry practice / about 10 minutes

Notice eating opportunities and access barriers without tracking quantities or weight.

No food challenge, weighing or calorie target. Ask your team privately about eating-related or medical concerns.

Section A - Entry practice Concept

Access, timing, health and medication instructions can affect eating routines. Explore what is useful for you with your team; this page does not prescribe meal intervals.

B - Eating opportunities and barriers

Day / setting	An eating opportunity	Barrier or support needed

Review eating-disorder, diabetes, medication, food-access, and cultural or religious needs privately with the relevant clinician. Choose another topic when appropriate.

C - Looking back

Would timing work be useful, or would food access, health or eating-related support be a better focus?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Describe an accessible eating opportunity; avoid food details if this topic is uncomfortable.

On another visit: Identify a food-access, cost, or scheduling barrier and a practical support request. Adapt eating-related work with your team, or skip it if uncomfortable or unsuitable for your care.

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PH-WS-03

An Evening Wind Down

Entry practice / about 10 minutes

Try a comfortable routine suited to your actual evening.

Section A - Techniques Menu

Comfortable slower breathing, if helpful; comfortable muscle relaxation; gentle movement. Stop if unwell or dizzy.

An option I would choose, adapt or skip, and why:

Section B - My 3-Step Evening Sequence

C - Looking back

What would you keep on a night with little time, privacy or space? An option can be skipped if it is unhelpful.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Point to a wind-down option or demonstrate a comfortable one; writing is optional.

On another visit: Build a brief wind-down option for a night with limited space, privacy, or time.

Ratings are optional. Skip them or use your own words. They aren’t a test or a diagnosis.

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PH-WS-04

Daily Demands and Energy

Extended practice / about 15 minutes

Use an energy-budget metaphor to consider demands and support.

Section A - My Current Demands

Demand	Cost (low/med/high)

Section B - Deposits vs. Withdrawals

Withdrawals (drain energy):

Deposits (restore energy):

C - Looking back

Which demand could be shared, simplified or deferred? Which cannot change and needs a different kind of support?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Draw two containers for demands and recovery, or describe one item in each.

On another visit: Choose one demand to simplify, defer, share, or discuss; some demands cannot be removed.

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PH-WS-05

Medication Routine and Questions

Entry practice / about 10 minutes

Prepare a question or reminder that follows your prescribed instructions.

Section A - Barriers to My Prescribed Routine

[ ] Side effects [ ] Cost / access [ ] Forgetting [ ] Unsure it helps [ ] Stigma [ ] Instructions unclear [ ] Other / N/A

A barrier I want to discuss:

Section B - My Reminder

Follow the prescribed dose, timing and food instructions. A habit reminder must fit those instructions, rather than change them.

Following my prescriber's instructions, I will use this reminder:

For a missed dose, use the medicine-specific instructions or contact your prescriber or pharmacist; do not guess. Do not start, stop or change medicine using this worksheet. Seek prompt clinical advice for urgent side effects or inability to obtain essential medicine.

C - Looking back

My question about a missed dose, side effect, access or instructions:

How will you reach the prescriber or pharmacist? Name a backup route for an unanswered important question.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Say a question for your prescriber; ask the team to help record it privately.

On another visit: plan how to reach the prescriber or pharmacist if an important question remains unanswered.

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PH-WS-06

Rest Withdrawal and What I Need

Extended practice / about 10 minutes

Explore context before judging whether rest or activity is useful.

Section A - A Flexible Self-Check

Rest and withdrawal can overlap. How I feel afterward is one clue, not a test. Consider illness, disability, demands, intention and my care plan; rest may be needed even if energy does not improve.

Section B - A Recent Example

What I did:

What I needed before, and how I felt afterward:

Closer to: [ ] Needed rest [ ] Withdrawal [ ] Mixed [ ] Unsure

C - Looking back

What need should guide the next choice: recovery, a manageable activity, an accommodation or a care conversation?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Point to rest, withdrawal, both, or unsure and name one need if you wish.

On another visit: Design a small re-entry option after rest without dismissing health or fatigue needs.

Rest and withdrawal are not moral categories. Illness, disability, overload and resources can matter; discuss what you need rather than judging effort.

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PH-SP-01

Warning Signs and Early Support

Extended practice / about 15 minutes

Notice general warning signs without retelling a crisis.

Section A - Warning Sign Categories

Thoughts / mood / behavior / physical sensations

Section B - My Early Warning Signs

Thoughts:

Mood:

Behavior:

Physical:

C - Looking back

If signs are hard to notice, what support route could you still use? Keep personal safety details for private care.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Name an early sign generally; do not share crisis details or access to means in group.

On another visit: Ask a trusted support how they might notice the sign; agree privately on a response.

If you can’t stay safe, or someone is in danger right now, get urgent help first. This page isn’t a safety plan.

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PH-SP-02

Coping Distraction and Asking for Help

Entry practice / about 10 minutes

Distinguish doing something alone, social distraction and directly asking for support.

Section A - Two Different Coping Options

Internal coping: a safe activity I can do on my own. Social distraction: a safe person or place that helps me shift attention. Asking someone directly for crisis help is a different step in my individualized safety plan.

Section B - My Options

Internal coping option:

Another internal option:

Safe person / place for social distraction:

A person I can tell directly that I need help:

C - Looking back

When would you skip coping practice and seek immediate help? Review that route privately with your clinician.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Point to an internal coping or social distraction option; contacts can be recorded privately.

On another visit: Distinguish distraction from directly asking for help; identify when support should take priority.

If you can’t stay safe, or someone is in danger right now, get urgent help first. This page isn’t a safety plan.

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PH-SP-03

A Private Environmental Safety Conversation

Entry practice / about 10 minutes

Arrange a private conversation about a safer environment.

Use with your clinician. Keep personal access or means details private; the group page is not an individualized safety assessment.

A - A private conversation I may need

You can ask for a safer-environment conversation without naming medicines, weapons, locations or access details on this group page.

B - Support and responsibility

Who will arrange or review the private plan with me?

Next action and agreed time; what help is needed to make it happen?

C - Follow-up

Who owns the next action, and when? An immediate concern needs help now, not a future appointment.

D - A question for my clinician

Without writing: Ask for a private conversation; no public hazard checklist is needed.

On another visit: Check privately whether the agreed action happened and whether the plan needs updating.

If you can’t stay safe, or someone is in danger right now, get urgent help first. This page isn’t a safety plan.

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PH-SP-04

An Urge and a Brief Pause

Entry practice / about 10 minutes

Use a manageable non-dangerous urge to explore a pause or another response.

Section A - A Flexible Wave Metaphor

Urges can rise and fall, but their duration varies. An urge may return or stay strong. You do not have to make it disappear to choose a safe response.

If an urge involves suicide, self-harm or harming someone else, or you may act on it, use your individualized safety plan and seek immediate support. Do not wait for a timer.

Section B - A Manageable Practice Example

A non-dangerous urge I can practice with:

Intensity: ___ /10 (0 = none; 10 = very strong). What changed over time?

A safe coping option I used, and what happened:

C - Looking back

What would you do if the urge stayed strong or the practice made things harder? Do not wait through a dangerous urge.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Describe an urge generally or point to intensity; observing the exercise is optional.

On another visit: Identify what to do if pausing is unhelpful. Safety concerns require urgent support, not waiting.

> If you can’t stay safe, or someone is in danger right now, get urgent help first. This page isn’t a safety plan.

Ratings are optional. Skip them or use your own words. They aren’t a test or a diagnosis.

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PH-SP-05

Responding to a Setback

Extended practice / about 15 minutes

Explore a response that can interrupt a giving-up pattern.

Section A - The Shame Cycle (reference)

Lapse > "I've already failed" thought > shame spike > "might as well continue" > fuller relapse

Section B - My Reframe

My typical self-critical response:

A more compassionate, course-correcting version:

C - Looking back

What care or support step is possible even if a reassuring thought does not feel believable?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Use a fictional setback to say a fair response and first support step.

On another visit: Make a course-correcting step possible even without motivation or a reassuring thought.

If you can’t stay safe, or someone is in danger right now, get urgent help first. This page isn’t a safety plan.

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PH-SP-06

Reviewing Safety Plan Access and Gaps

Entry practice / about 15 minutes

Privately review access, contacts and missing parts of your clinician-developed plan.

Section A - Review My Individualized Safety Plan

[ ] I know my warning signs [ ] I have safe internal coping options [ ] I have people / places for social distraction [ ] I know whom I can tell directly that I need help [ ] Professional / urgent contacts are current [ ] Environmental safety was discussed privately with my clinician

Where my individualized plan is kept:

Section B - Verified Contacts

Person I can directly ask for help / phone:

Clinician / program and after-hours contact:

Local crisis service / emergency number:

If a contact does not answer, my next appropriate contact is:

C - Looking back

Which item is unclear, outdated or missing? Agree with the clinician who will address it and when.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Arrange private help to check contacts and access. You do not need to read names or numbers to the group.

On another visit: Check the backup contact and after-hours route with your team; do not make test emergency calls.

If you can’t stay safe, or someone is in danger right now, get urgent help first. This page isn’t a safety plan.

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PH-TS-01

Questions About Stepping Down

Entry practice / about 10 minutes

Prepare questions for your team; this page does not decide readiness.

Section A - General Factors Teams Often Consider (reference)

Symptom stability / safety / daily functioning / support system / skill engagement

Section B - My Questions for My Treatment Team

C - Looking back

Who can answer one unresolved question, and how will you reach them? Attendance or a score cannot decide discharge.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that's too much: an easier option, or someone who can help:

Without writing: Say one question for the team and ask for help recording it; this page cannot decide discharge.

On another visit: Identify who can answer an unresolved question and when you will ask.

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PH-TS-02

Life Roles Beyond Treatment

Extended practice / about 15 minutes

Explore a role you choose, rather than one you are expected to resume.

Section A - Life Roles Menu

Worker/student / parent/family member / friend / partner / community member / hobbyist/creative

Section B - My Roles Right Now

Which feel active, dormant, or changed:

A role I choose to continue, change, begin or leave:

C - Looking back

What smaller version fits your health, responsibilities and supports now? A role can also be changed or declined.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Point to a life role or draw it; you can keep personal details private.

On another visit: Choose a smaller version of a role that fits your current health and support.

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PH-TS-03

My Routine and Check In Plan

Extended practice / about 15 minutes

Choose a brief routine or check-in that is useful outside the program.

A - What my program actually provides

Possible supports to check with the team: structure, check-ins, group contact and staff access. Which are available now, which continue after discharge, and what are their limits?

Section B - My Self-Monitoring System

What I'll track:

How often:

How (tool/method):

C - Looking back

What is the restart plan after a missed check-in? What support remains available, and for how long?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Describe a check-in routine and a reminder; ask a support person to help record it if appropriate.

On another visit: Build a restart plan for a missed check-in rather than abandoning the whole routine.

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PH-TS-04

Follow Up and Handoff Questions

Entry practice / about 15 minutes

Check that the next provider and practical access arrangements are confirmed.

Section A - Provider Fit

[ ] Communication [ ] Approach [ ] Availability [ ] Cost [ ] Accessibility / language [ ] Other:

Section B - First-Visit Summary

What I want the provider to know (briefly):

One question I want to ask:

Your own summary is optional and does not replace the team's authorized clinical handoff. Confirm who sends essential information, to whom, through which approved route, and when.

C - Looking back

Private follow-up check: provider / contact / date:

Confirmed? [ ] Yes [ ] Not yet. Cost / transport / access / language help needed:

Bridge or backup contact and availability to confirm with the team:

Who will transfer essential information through the agreed private route? What is the bridge plan if care is delayed?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Say one handoff question privately; ask for help recording provider and access details.

On another visit: Choose an action if a referral is not accepted or an appointment is delayed; confirm with the team.

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PH-TS-05

Mixed Feelings About Leaving

Extended practice / about 15 minutes

Explore mixed feelings alongside the practical transition plan.

Section A - Transition Feelings Menu

Relief / fear of regression / grief about ending relationships here / guilt about "still needing more" / excitement / uncertainty

Section B - My Feelings, Rated

Feeling	Intensity (0-10)

C - Looking back

What needs emotional support, and what needs a practical task? Neither a feeling nor this page decides readiness.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Point to relief, worry, sadness, mixed, or unsure; explaining is optional.

On another visit: Notice which concern needs emotional support and which needs a practical plan.

Ratings are optional. Skip them or use your own words. They aren’t a test or a diagnosis.

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PH-TS-06

My Recovery Reference

Entry practice / about 20 minutes

Keep a short reference you could use when tired or distracted.

Section A - My Useful Skills

Skill 1 and where / how I use it:

Skill 2 and where / how I use it:

Section B - My Plan and Support

Location of my individualized safety plan:

A meaningful direction or value (see PE-10):

Support person / service (see PE-16 and PH-TS-04):

C - Looking back

Early warning sign / support step / backup when routine support is unavailable:

Can you find the skill, support or follow-up detail you need? Remove clutter and verify anything that has changed.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Talk through one useful plan element; your team can help make a private accessible copy.

On another visit: Check whether the summary is usable when tired; simplify one part and verify the support route.

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PH-AP-01

Practicing a Request or Boundary

Extended practice / about 20 minutes

Rehearse a manageable, safe script or observe a demonstration.

Choose a fictional or low-risk situation. Ask before giving feedback; anyone may decline or stop. Do not rehearse confronting an unsafe person.

Section A - My Real Situation

Answer:

Section B - My Script Draft, and Revision After Feedback

Draft:

Revised:

Choose a low-stakes or fictional situation. Ask permission before feedback; real-world threats or coercion need private planning.

C - Looking back

What would you say if the request is declined? Consent to feedback and real-world safety matter more than performance.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Rehearse a brief request using a fictional situation, or observe a facilitator demonstration.

On another visit: Try a safe response to a refusal and identify what you can control next.

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PH-AP-02

Noticing and Grounding

Extended practice / about 15 minutes

Choose an external cue or an already-present mild sensation; no symptoms are deliberately induced.

Section A - Safe Scope and Choice

Notice a naturally occurring, mild sensation while seated or otherwise supported. Do not deliberately create symptoms with breath-holding, rapid breathing, spinning or strenuous exercise. You may observe, focus on an external object, or pass.

Tell your facilitator about any health or mobility concern. Stop if you feel dizzy, unwell, more distressed or unsafe. This is not exposure therapy or a test of endurance.

Section B - My Observation

A mild sensation, or an external cue I chose:

Intensity: ___ /10 (0 = none; 10 = very strong):

A comfortable regulation or grounding option:

What happened, including no change:

C - Looking back

Would you use, adapt or stop this option? New or concerning physical symptoms need medical attention.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Point to an external grounding option; body-focused practice can be skipped.

On another visit: Compare external grounding with a comfortable option; no reduction in distress is required.

Ratings are optional. Skip them or use your own words. They aren’t a test or a diagnosis.

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PH-AP-03

Working With a Manageable Thought

Extended practice / about 15 minutes

Work with a current thought that is suitable for general group practice.

Section A - My Current Thought

Answer (general terms okay):

Original thought before: ___ /10 (0 = not believable; 10 = completely believable):

Section B - Evidence For / Against

Facts supporting the thought	Other facts or missing context

Do not use this group record to debate delusions or hearing voices. Ask your clinician for an adapted individual approach when needed.

C - Looking back

A fairer, fuller alternative that includes the available facts:

Re-rate the original thought only if useful: 0 = not believable; 10 = completely believable. The rating may stay the same.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Talk through a manageable thought or use a fictional example with facilitator support.

On another visit: If the thought does not shift, identify useful action or individual follow-up instead of forcing it.

Ratings are optional. Skip them or use your own words. They aren’t a test or a diagnosis.

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PH-AP-04

Trying a Small Task Step

Entry practice / about 15 minutes

Test a feasible first move or identify the help needed before trying.

Choose a manageable task with your facilitator. You may change, pause or stop it and ask for support.

Section A - My Stuck Task

The task I am considering:

Section B - My Tiniest First Step

The part I can draft or begin here:

What happened when I attempted it: [ ] Completed [ ] Partially [ ] Not yet - what got in the way:

Use a draft or reversible practice. Do not send a message, spend money, share credentials or make a consequential commitment merely to complete the activity.

C - Looking back

What happened: tried, partly tried, not tried, changed plan or unsure? What should change before another attempt?

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Say one small task step and one obstacle; the facilitator may draw the plan.

On another visit: Test a step with a realistic resource or energy limit, then reduce it or request support.

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PH-AP-05

Finding My Safety Plan

Entry practice / about 10 minutes

Practice access when calm, privately and without making test emergency calls.

Section A - Locate and Review

Notice a warning sign > Locate my individualized plan > Identify the appropriate next step > Check how I would access that support.

I do not have to start with internal coping if immediate support is needed. If I cannot stay safe, I contact urgent support right away.

Section B - A Calm Practice Check

Where is my plan, and can I access it?

An appropriate contact / support step and how I would reach it:

Any obstacle (phone, storage, accessibility, outdated contact):

C - Looking back

Can you reach an accessible backup if the usual copy or phone is unavailable? Agree on the fix with your team.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that’s too much: an easier option, or someone who can help:

Without writing: Show where your plan is kept privately; you do not need to read its contents to the group.

On another visit: Check access in another safe setting and choose an accessible backup copy with your team.

If you can’t stay safe, or someone is in danger right now, get urgent help first. This page isn’t a safety plan.

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PH-AP-06

A Skill Outside the Group

Extended practice / about 15 minutes

Plan for a real setting that differs from the treatment room.

Section A - My Skill and My Real Environment

Skill:

Environment (home/work/other):

Section B - The Gap: Here vs. There

Here (this program)	There (my real environment)

C - Looking back

Which suggestion, if any, do you choose to use? Feedback is optional; name a cue and a realistic backup.

D - My next step

One thing to try, notice, or ask (optional), and what will remind me:

If that's too much: an easier option, or someone who can help:

Without writing: Describe one real-world setting and a skill choice; a spoken rehearsal can replace writing.

On another visit: Plan for a likely interruption and a restart cue; agree on safe difficulty with the facilitator.

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Form 1 / The Rotation Passport

Bring this to group if you find it useful. Your facilitator records attendance the way the program requires.

Optional program-approved identifier:

Admission date:

Program: [ ] IOP [ ] PHP

Process Group (PR)

Code	Title	Date attended	Facilitator
PR-01	Arriving		
PR-02	Carrying the Week		
PR-03	The Mask and the Mirror		
PR-04	Frustration and Stuckness		
PR-05	Trust and Safety Here		
PR-06	The Inner Critic		
PR-07	Grief Loss and What Changed		
PR-08	Hope and Doubt		
PR-09	Connection and Isolation		
PR-10	Resilience and Quiet Wins		
PR-11	Giving and Receiving Feedback		
PR-12	Anger and Boundaries		
PR-13	Shame and Self Forgiveness		
PR-14	Identity Beyond the Diagnosis		
PR-15	Navigating Outside Relationships		
PR-16	Setbacks and Recovery Momentum		
PR-17	Preparing for Transition		
PR-18	Endings Goodbyes and Continuity		

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Form 1 / Rotation Passport - Skills sessions

Code	Title	Date attended	Facilitator
SK-MF-01	Observe and Describe		
SK-MF-02	My Grounding Options		
SK-MF-03	Noticing and Returning Attention		
SK-ER-01	Emotions Urges and Actions		
SK-ER-02	Check the Facts and Choose a Response		
SK-ER-03	Routines That Support Me		
SK-DT-01	Coping Through an Intense Moment		
SK-DT-02	Acceptance and My Next Choice		
SK-DT-03	Comfort and Distraction Options		
SK-CG-01	Situation Thought Feeling Action		
SK-CG-02	A Fuller Thought Record		
SK-CG-03	Making Room Around a Thought		
SK-BA-01	Activities Mood and Energy		
SK-BA-02	A Value and a Small Action		
SK-BA-03	My Task Steps and Reminder		
SK-IE-01	Making a Clear Request		
SK-IE-02	A Boundary I Can Keep		
SK-IE-03	Saying No and Understanding Guilt		

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Form 1 / Rotation Passport - Psychoeducation sessions

Code	Title	Date attended	Facilitator
PE-01	Depression and Withdrawal		
PE-02	Anxiety and the Alarm Response		
PE-03	Stress Demands and Recovery		
PE-04	Understanding a Panic Cycle		
PE-05	Sleep Habits and Mood		
PE-06	Strong Emotions and a Pause		
PE-07	A Guide to Feeling Able to Engage		
PE-08	Trauma Reminders and Present Support		
PE-09	A Fairer Response to Self Criticism		
PE-10	Values and Everyday Choices		
PE-11	Understanding Anger and Responding		
PE-12	Grief Loss and Support		
PE-13	Loneliness and Connection		
PE-14	Boundaries in Relationships		
PE-15	When Starting a Task Is Hard		
PE-16	Support Roles and Help Routes		
PE-17	Setbacks and Earlier Support		
PE-18	Follow Up After Stepping Down		

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Form 1 / Rotation Passport - PHP extension and special protocols

Code	Title	Date attended	Facilitator
PH-WS-01	Daily Rhythm Anchors		
PH-WS-02	Eating Opportunities and Energy		
PH-WS-03	An Evening Wind Down		
PH-WS-04	Daily Demands and Energy		
PH-WS-05	Medication Routine and Questions		
PH-WS-06	Rest Withdrawal and What I Need		
PH-SP-01	Warning Signs and Early Support		
PH-SP-02	Coping Distraction and Asking for Help		
PH-SP-03	A Private Environmental Safety Conversation		
PH-SP-04	An Urge and a Brief Pause		
PH-SP-05	Responding to a Setback		
PH-SP-06	Reviewing Safety Plan Access and Gaps		
PH-TS-01	Questions About Stepping Down		
PH-TS-02	Life Roles Beyond Treatment		
PH-TS-03	My Routine and Check In Plan		
PH-TS-04	Follow Up and Handoff Questions		
PH-TS-05	Mixed Feelings About Leaving		
PH-TS-06	My Recovery Reference		
PH-AP-01	Practicing a Request or Boundary		
PH-AP-02	Noticing and Grounding		
PH-AP-03	Working With a Manageable Thought		
PH-AP-04	Trying a Small Task Step		
PH-AP-05	Finding My Safety Plan		
PH-AP-06	A Skill Outside the Group		

Special Protocols (SP)

Code	Title	Date attended	Facilitator
SP-OR	Program orientation		
SP-SD	Private transition review and optional goodbye		
SP-CD	Support after a difficult group event		
SP-AV	Low attendance or program disruption		

Optional program ID: _____ Passport page 4 of 5

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Form 1 / Rotation Passport: Coverage & Review

The Passport records attendance for the codes your team assigns. You don't need to complete all 82.

Assigned coverage

Assigned applicable codes: _____ Different assigned codes attended: _____

Count each code once, even if you attended it more than once. Coverage = different assigned codes attended ÷ assigned codes × 100. If no codes are assigned, write N/A. Record repeat visits separately if the program needs them.

What attendance does not decide

Attendance doesn't show that a skill is being used, that someone is safe, or that they're ready to leave. There is no pass mark.

Review with my team

A skill I understand or have tried, and support still needed:

Relevant missed topics and agreed review or follow-up:

Reviewed with client on: _____ Staff / clinician: _____

Next action / owner / agreed time:

Personal Crisis and Support Card

Print single-sided at actual size (100%). Cut the outer dashed border, then fold on the center line to make one 3.5 x 2.25 inch card. Page 98 is intentionally blank for the reverse. Use 124 for a larger contact sheet.

Verify local contacts privately before use. If you cannot stay safe or there is immediate danger, seek urgent help. This card does not replace your individualized safety plan.

IF I NEED URGENT HELP

Immediate danger: local emergency services

US: call / text 988 (Suicide & Crisis Lifeline)

Other country: verified local services

Local emergency: _____

Local crisis service: _____

Program after-hours: _____

If no answer, next: _____

MY SUPPORT CONTACTS

Person I can tell I need help: _____

Phone: _____

Clinician / program: _____

Phone / hours: _____

My safety plan is kept: _____

One coping option: _____

Verified with / date: _____

Measure the folded card if printing from a copy. Keep the blank reverse and do not reduce its scale. Choose the full-page format if the card text or small writing spaces are difficult to use.

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Back of the Folded Card

This page is the back of the folded card on page 97, left blank so you can copy the card single-sided. Nothing is missing. For more writing space, use the full-page contacts on page 124.

Skills Review / Mindfulness and Grounding

A short review chosen with your facilitator. It doesn't replace the session.

SK-MF-01 / Observe and Describe

Describe one observable detail. Notice a judgment without treating it as a fact.

SK-MF-02 / My Grounding Options

Choose an external sight, sound, texture or support point that is available and comfortable. Grounding may not feel calming; adapt or stop if it makes things harder.

SK-MF-03 / Noticing and Returning Attention

Notice attention wandering and return to a chosen cue. Use a shorter practice or another cue if needed.

Choose one practice

Describe an ordinary object without evaluating it, or observe a demonstration.

A question, or something that got in the way:

What I'll try or ask next, and what will remind me:

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Skills Review / Emotion Regulation

A short review chosen with your facilitator. It doesn't replace the session.

SK-ER-01 / Emotions Urges and Actions

An emotion can come with an urge; your action may follow it or differ. Pairings on a reference table are examples, not rules.

SK-ER-02 / Check the Facts and Choose a Response

Check facts, intensity, safety and effectiveness. Consider opposite action, problem-solving or support; do not use opposite action to ignore real danger.

SK-ER-03 / Routines That Support Me

Consider sleep, food access, physical health, activity, substance-related care and accumulated demands. Resources and accommodations may matter more than effort.

Choose one practice

Choose one situation and distinguish an urge from an action, using a fictional example if useful.

A question, or something that got in the way:

What I'll try or ask next, and what will remind me:

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Skills Review / Distress Tolerance

A short review chosen with your facilitator. It doesn't replace the session.

SK-DT-01 / Coping Through an Intense Moment

Choose a comfortable coping option for a manageable practice. A coping exercise must never delay urgent help.

SK-DT-02 / Acceptance and My Next Choice

Acknowledge a fact without approving it or staying in danger. Keep protective action available.

SK-DT-03 / Comfort and Distraction Options

Sensory comfort or distraction may help for now; an underlying problem may still need practical help or care.

Choose one practice

Try or observe one comfortable option, then choose an alternative if it is unhelpful.

A question, or something that got in the way:

What I'll try or ask next, and what will remind me:

If you cannot stay safe, seek urgent help rather than waiting for a skill to work.

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Skills Review / Cognitive Skills

A short review chosen with your facilitator. It doesn't replace the session.

SK-CG-01 / Situation Thought Feeling Action

Explore possible links among a situation, thought, feeling and action. A thinking-trap label is optional; real-world threats and barriers also matter.

SK-CG-02 / A Fuller Thought Record

Use available facts to form a fuller view. A thought or feeling is not required to change.

SK-CG-03 / Making Room Around a Thought

Try a noticing phrase for a manageable thought, such as I am having the thought that. This does not decide whether the thought is true.

Choose one practice

Use a fictional thought to add missing context, then choose a useful action.

A question, or something that got in the way:

What I'll try or ask next, and what will remind me:

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Skills Review / Behavioral Activation

A short review chosen with your facilitator. It doesn't replace the session.

SK-BA-01 / Activities Mood and Energy

Notice activities, meaning, enjoyment, effort and energy. Choose a manageable activity without demanding an immediate mood improvement.

SK-BA-02 / A Value and a Small Action

Choose a meaningful direction and an action that fits health, safety and resources. Delaying an unsafe task is not a failure.

SK-BA-03 / My Task Steps and Reminder

Break a task into usable parts and connect the next part to a cue. A practical barrier may need outside support.

Choose one practice

Name the next move in one task, a cue to start and any resource needed first.

A question, or something that got in the way:

What I'll try or ask next, and what will remind me:

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Skills Review / Requests and Boundaries

A short review chosen with your facilitator. It doesn't replace the session.

SK-IE-01 / Making a Clear Request

Describe a situation, express your view, make a request and explain why it matters. This four-part practice is not the full DEAR MAN skill.

SK-IE-02 / A Boundary I Can Keep

Acknowledge another perspective if useful and state a limit about your own choices. You need not validate harm.

SK-IE-03 / Saying No and Understanding Guilt

Practice a brief decline and consider whether guilt points to a repair, discomfort, pressure or several things.

Choose one practice

Rehearse a request or decline in a fictional safe situation, including what to do if the answer is no.

A question, or something that got in the way:

What I'll try or ask next, and what will remind me:

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My Bridge From Group to Everyday Life

Optional. Choose one skill and one manageable place to use it, with your team.

1. What matters to me

A meaningful direction or everyday need; a word or drawing is enough.

---

2. The situation and cue

Where could I use the skill? What might remind me? Avoid identifying details about others.

---

3. My smallest feasible action

One action I could say, show or do, with support if needed.

---

4. A realistic barrier and easier option

Consider energy, health, cost, transport, privacy, language, housing or caregiving. What can be adapted; what needs help?

---

5. Support and follow-up

Who / which service could help? What is the next action, who owns it, and when? Arrange sensitive details privately.

---

6. Check the explanation

In my own words or by showing: when the cue happens, what will I do? If the first option is unavailable, what is my backup?

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What Happened When I Tried?

A quick look at how it went. It isn't a symptom score or a test for discharge.

The plan I chose

Code / page: _____ Step / reminder: _____

What happened?

[ ] Tried [ ] Partly tried [ ] Did not try [ ] Chose another option [ ] Unsure What I noticed, in a word, drawing or short account:

What was useful or unhelpful?

A skill can be useful even if your mood doesn't change right away. No change is useful to know, too.

What got in the way?

[ ] Unclear step [ ] Forgot / no cue [ ] Too much effort [ ] Health / energy [ ] Cost / resources [ ] Transport / access [ ] Privacy / language [ ] Did not fit my needs [ ] Something else [ ] Prefer private discussion

What adjustment fits?

[ ] Keep it [ ] Make it smaller [ ] Change the cue [ ] Change the skill [ ] Ask for practical support [ ] Pause and discuss with the team My next action / reminder / backup:

What do I want the team to know?

Support requested / person responsible / agreed timeframe:

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Review Routes - Psychoeducation 01-09

Choose topics that matter for your care, with your team. A review isn't the same as attending. You can speak, draw, watch, or pass.

PE-01 / Depression and Withdrawal (29)

Low mood can affect activity, connection, and routine; this page does not diagnose depression. Try: Choose one manageable activity or support request, even if mood stays the same. Discuss: What could make that step easier to begin?

PE-02 / Anxiety and the Alarm Response (30)

Anxiety has several signs; a sensation alone does not establish its cause. Try: Name one familiar sign and a response discussed with your team. Discuss: When would you seek medical advice instead of assuming anxiety?

PE-03 / Stress Demands and Recovery (31)

Demands and recovery opportunities may be uneven; many demands are outside your control. Try: Identify one demand and one realistic support, boundary, or recovery opportunity. Discuss: Which barrier needs practical help rather than more effort?

PE-04 / Understanding a Panic Cycle (32)

A familiar cycle can link sensations, frightening interpretations, and protective behaviors. Try: Use a fictional example to identify one link; choose a response with your clinician. Discuss: Why should new, severe, or unusual symptoms receive medical attention?

PE-05 / Sleep Habits and Mood (33)

Routine habits may support sleep; they are not a complete treatment for chronic insomnia. Try: Choose one feasible habit and adapt it to work, caregiving, housing, or health. Discuss: What ongoing sleep concern would you bring to your team?

PE-06 / Strong Emotions and a Pause (34)

Strong emotions can make reflection harder; the model is simplified education. Try: Choose a brief pause cue or a support request; use an external cue if internal signs are unclear. Discuss: What would your cue prompt you to do next?

PE-07 / A Guide to Feeling Able to Engage (35)

The three-zone guide describes experience; it does not measure a nervous-system state. Try: Identify a comfortable condition that makes engagement easier today. Discuss: What adaptation would help if the exercise is uncomfortable?

PE-08 / Trauma Reminders and Present Support (36)

Present-focused support can be discussed without recounting traumatic events. Try: Choose a grounding option, a pause signal, or a private support request. Discuss: How could you ask for help without explaining the event in group?

PE-09 / A Fairer Response to Self Criticism (37)

A fair, compassionate response can acknowledge difficulty without promising an outcome. Try: Turn one general self-judgment into a specific, kinder statement. Discuss: Does the statement feel believable; how could you make it more modest?

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Review Routes - Psychoeducation 10-18

Choose topics that matter for your care, with your team. A review isn’t the same as attending. You can speak, draw, watch, or pass.

PE-10 / Values and Everyday Choices (38)

A value is a meaningful direction; an action can be very small and adapted. Try: Name one value and a low-cost action suited to your energy today. Discuss: How could the action change while the value stays the same?

PE-11 / Understanding Anger and Responding (39)

Anger may point to a problem or boundary; the urge is not the only possible response. Try: Identify one early sign and a safe problem-solving or support option. Discuss: What can you control without ignoring danger or a necessary boundary?

PE-12 / Grief Loss and Support (40)

Loss may involve people, roles, health, or hopes; there is no required grief timetable. Try: Choose a welcome support or a small routine; passing on personal details is okay. Discuss: What kind of support would you welcome or decline right now?

PE-13 / Loneliness and Connection (41)

Connection can be brief and low pressure; not all available contacts feel safe. Try: Choose a manageable connection or a practical access support. Discuss: What could you try if time, money, transport, or energy is limited?

PE-14 / Boundaries in Relationships (42)

A boundary identifies your limits and choices; it does not control another person's behavior. Try: Rehearse a boundary using a fictional, safe example. Discuss: What support would you seek if stating the boundary is unsafe?

PE-15 / When Starting a Task Is Hard (43)

Difficulty starting may involve unclear steps, emotion, fatigue, health, or resource barriers. Try: Choose one task and identify its smallest actionable first step. Discuss: Does the barrier need a smaller step, information, accommodation, or resources?

PE-16 / Support Roles and Help Routes (44)

Social contact, treatment help, and urgent crisis support have different roles. Try: With your team, verify one contact and a backup privately. Discuss: Which route would you use if you cannot stay safe or the first contact is unavailable?

PE-17 / Setbacks and Earlier Support (45)

A setback can prompt review and support; it does not erase previous effort. Try: Name a general early sign and a support action suited to your care plan. Discuss: What would help you ask for support sooner?

PE-18 / Follow Up After Stepping Down (46)

Transition planning includes access and support; attendance alone cannot decide readiness. Try: Identify a follow-up need, appointment question, or access barrier for the team. Discuss: What is the backup if the first follow-up arrangement fails?

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Review Routes - PHP Routines and Support

Choose topics that matter for your care, with your team. A review isn't the same as attending. You can speak, draw, watch, or pass.

PH-WS-01 / Daily Rhythm Anchors (67)

Try: Choose one feasible anchor and track it briefly if useful. Discuss: How would you adapt it on a disrupted day?

PH-WS-02 / Eating Opportunities and Energy (68)

Try: Identify one accessible eating opportunity or a request for food support. Discuss: What barrier needs help; would you prefer to skip food-focused work?

PH-WS-03 / An Evening Wind Down (69)

Try: Choose a comfortable option requiring little time or equipment. Discuss: What would you use if your usual space or routine is unavailable?

PH-WS-04 / Daily Demands and Energy (70)

Try: Choose one demand to simplify, share, defer, or discuss. Discuss: Which demand is outside your control and needs support?

PH-WS-05 / Medication Routine and Questions (71)

Try: Prepare one question and how you will reach the appropriate professional. Discuss: What would you do if the question remains unanswered?

PH-WS-06 / Rest Withdrawal and What I Need (72)

Try: Notice the context and choose rest, a small re-entry, or a care conversation. Discuss: What information or accommodation would help you decide?

PH-SP-01 / Warning Signs and Early Support (73)

Try: Name one sign generally and arrange private review with your team. Discuss: Who could help you notice it and what response have you agreed?

PH-SP-02 / Coping Distraction and Asking for Help (74)

Try: Identify one comfortable option and a route for directly asking for support. Discuss: When should urgent support take priority over coping practice?

PH-SP-03 / A Private Environmental Safety Conversation (75)

Try: Arrange or follow up that conversation; do not record means-access details here. Discuss: If safety is immediate, why should you seek help now rather than wait?

PH-SP-04 / An Urge and a Brief Pause (76)

Try: Try or observe a manageable pause only if appropriate, then identify support. Discuss: What is your alternative if the practice is uncomfortable or unhelpful?

PH-SP-05 / Responding to a Setback (77)

Try: Choose a small course-correcting action suited to your individual care plan. Discuss: What action is possible even if the reassuring thought does not feel true?

PH-SP-06 / Reviewing Safety Plan Access and Gaps (78)

Try: Privately verify current contacts, after-hours support, and your plan location with the team. Discuss: What is the agreed backup if a contact does not answer?

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Review Routes - PHP Transition and Application

Choose topics that matter for your care, with your team. A review isn't the same as attending. You can speak, draw, watch, or pass.

PH-TS-01 / Questions About Stepping Down (79)

Try: Choose an unanswered transition question and identify who can answer it. Discuss: When and how will you ask; what help do you need to access the answer?

PH-TS-02 / Life Roles Beyond Treatment (80)

Try: Choose a role and a modest, feasible action. Discuss: What would make that role accessible rather than overwhelming?

PH-TS-03 / My Routine and Check In Plan (81)

Try: Choose what to track, how briefly, and when; keep it private as appropriate. Discuss: What is your restart step if you miss a check-in?

PH-TS-04 / Follow Up and Handoff Questions (82)

Try: Verify one next-provider question and the owner of the next action. Discuss: Who will help if there is a delay, rejection, cost, or transport barrier?

PH-TS-05 / Mixed Feelings About Leaving (83)

Try: Identify one emotional support need and one practical planning need. Discuss: Which concern needs a conversation and which needs an access task?

PH-TS-06 / My Recovery Reference (84)

Try: Review one routine, support route, or follow-up detail privately. Discuss: Could you use the summary when tired; what should be simplified?

PH-AP-01 / Practicing a Request or Boundary (85)

Try: Rehearse one sentence, or observe a demonstration and choose a phrase. Discuss: What could you do if the request is declined?

PH-AP-02 / Noticing and Grounding (86)

Try: Try or observe a comfortable option; stop if more distressed or physically uncomfortable. Discuss: What alternative would you choose without requiring your distress rating to improve?

PH-AP-03 / Working With a Manageable Thought (87)

Try: Use a manageable thought or fictional example and consider another perspective. Discuss: If belief does not shift, what useful action or individual follow-up could still help?

PH-AP-04 / Trying a Small Task Step (88)

Try: Test the smallest feasible step and identify a barrier before increasing difficulty. Discuss: Would a smaller step, resource, accommodation, or support request fit better?

PH-AP-05 / Finding My Safety Plan (89)

Try: Locate the plan or accessible copy; do not make test emergency calls. Discuss: What backup copy or access route have you agreed with your team?

PH-AP-06 / A Skill Outside the Group (90)

Try: Choose one safe setting, one skill, one cue, and an easier alternative. Discuss: What would you do if the first attempt is interrupted or does not help?

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Return After Absence: Review, Then Reconnect

A summary can’t recreate a group you missed. Decide with your team which follow-up matters for your care.

A topic I missed

Code / page: _____ Is it relevant to my current care? _____ [ ] SK summary (100-105) [ ] PE route (116-117) [ ] PHP route (118-119) [ ] PR reconnection below [ ] Facilitator-led special protocol review

PR reconnection: join the group that exists today

Ask for a brief orientation to the topic and frame. In a word, symbol or privately: what would help me rejoin? What concern needs a private conversation? Listen or choose one optional prompt on the relevant PR page. Do not ask another member to retell an identifiable story.

Special protocols: decide with the facilitator

The Passport lists SP-OR, SP-SD, SP-CD and SP-AV. These sessions are led by your facilitator, so plan any review with them. After a critical event, nobody should be asked to retell it, and psychological debriefing is not used to prevent PTSD (NICE NG116, recommendation 1.6.5).

Check understanding and name the next action

One idea I can say or show: _____ Question still open: _____ Support / follow-up owner / when: _____

Sources

The explain-it-back approach follows AHRQ Tool 5; group privacy follows APA Standards 4.02 and 10.03; the limit on psychological debriefing follows NICE NG116, recommendation 1.6.5. See page 121.

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My Crisis and Support Contacts

Complete and verify these details privately with your team. This contact sheet is not the individualized safety plan. If you cannot stay safe or there is immediate danger, seek urgent support now.

United States: call or text 988 / Suicide & Crisis Lifeline. Elsewhere: use verified local crisis and emergency contacts.

Local emergency number and when to use it

Local crisis service

Person I can tell directly that I need help / contact

Clinician or program / contact and hours

After-hours route

If no answer, next appropriate contact

Where I keep my individualized safety plan / accessible backup

---

One coping option I have chosen with my team

Verified with / date / next review

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Topic Finder / Ask Your Facilitator

Use this to ask your facilitator about a topic. It isn't for choosing a safety or trauma exercise on your own; your facilitator matches pages to your care plan.

If you want to discuss	Possible starting topics to ask about
Getting oriented or reconnecting	PR-01 / PR-09 / SP-OR with staff
Attention and grounding	SK-MF-01 / SK-MF-02 / SK-MF-03
Emotions and urges	SK-ER-01 / SK-ER-02 / SK-DT-01
Thoughts and practical action	SK-CG-01 / SK-CG-02 / SK-BA-03
Requests and boundaries	SK-IE-01 / SK-IE-02 / PH-AP-01
Rest, demands and everyday routines	PH-WS-01 / PH-WS-04 / PH-WS-06
Transition and follow-up	PH-TS-01 / PH-TS-04 / PH-TS-06

If you need urgent help, tell staff or use your urgent-help contact now. Don't look for a worksheet first.

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